CHAPTER-1

INTRODUCTION

Healthcare administrators have long recognized the importance of patient satisfaction as a key performance indicator. Several factors contribute to this indicator, including but not limited to: responsiveness, communication, attitude, clinical skill, amenities, food services, and admission and discharge services. Patients' evaluations of hospital treatment reportedly take into account not one but two distinct aspects of health care providers' interpersonal and technical competence.

Customer satisfaction can only be sustained with the use of an accurate approach for tracking it. In order to set standards for future evaluations, organizations must continually track the customer-related impacts of management decisions.

A person's broad opinion regarding their overall health care experiences is known as patient satisfaction. It is attained when patients feel that the services and attention they get from medical professionals are of high quality. The environment has met their expectations and been good and fulfilling. Involving patients or customers in the administration of their care and treatment is one of the major themes in the evolution of contemporary health care. Person-centered healthcare upholds the worth and dignity of every individual. It is perfectly appropriate and good to find out what patients think about their expectations and experiences with medical treatment.

Patients' feedback is now gathered by modern companies since it may impact the whole quality improvement agenda and provide a chance for organizational learning and growth. Similar to many other psychological notions, satisfaction is simple to grasp but difficult to quantify. Rather than being a fixed occurrence, individuals describe satisfaction as a judgment that they acquire over time as they think back on their experiences.

INDUSTRY PROFILE AND HOSPITAL PROFILE:

PROFILE OF INDIAN HEALTHCARE INDUSTRY

A hospital is a kind of medical facility that offers treatment to patients by means of trained medical professionals, ancillary healthcare workers, and advanced medical technology. The most comman kind of hospitel is thegeneral hospitel, whichusually features an emergncy roomto handle people with unexpected illnesses, burn victims, and other serious accidents. With a large number of critical care beds and extra beds for patients requiring long-term care, a districthospital is usualy the main health care center in its area. There are specialised hospitalsthat cater to certain medical requirements, such as those associated with mental treatment (for further information, see psychiatric hospital) or specific disease categories. These hospitalsmay also have a focus on rehabilitation, children's care, the elderly (geriatric), or trauma. Health care expenditures may be reduced with the aid of specialized hospitals as opposed to ordinary hospitals. Based on their funding sources, hospitals may be categorized as either general, specialized, or government.

Helping patients is only one part of what ateaching hosptal does; they also educate future doctrs, nursas, and othermedical professionals. Clinics are smaller health science facilities compared to hospitals. Cardiology is one of several specialized divisions found in hospitals, among others like surgery and urgent care. Chronic treatment units and outpatient sections are two servicesoffered by various hospitals. It is common to see a pharmacy,pathology, and radiologysupport unit.

Most hospitals get financing from the government, privatehealth insurace companis, nonprofit organizations, for-profithealth organisations, or charity contributions. Religious organizations or generous citizens and influential people have established and supported hospitalin the past.

Professionel doctor, surgeon, nurseand otherhealth care providers now make up the majority of the hospital personnel, as opposed to the past when volunteers or members of founding religious organizations handled this task. Nonetheless, a number of other Christian faiths, such as the Methodiests and Lutheraans, as wellas a number of Catholic religious orders, such as the Aleians and theBon SecoursSisters, continued to concentrate on hospital minitry in the late 1990s.[4] Accordng tothe word's original definition, hospitals were "places of

hospitality," and certain institutions' titles still reflect this original meaning. One such example is the RoyalHospital Chelsea, which was fouded in 1681 as a retirment and nursing homefor formersoldiers.

A hospitalmay accept certain patients ("admitted"), who remain overnightor for many days, weeks,or months ("inpatients"), while other patients visit the facility just for diagnostic, treatment,or therapyand then depart ("outpatients"). The capacity of hospitals to accept and carefor inpatients sets them apart from other kinds of medical institutions; smaller facilities are more often referred to as clinis.

GENERAL ANDACUTE CARE

Among hospitals, themost well-known is thegeneral hospital, often called anacute-care hospital. Emergency rooms (also called "accident & emergency") or trauma centers are common at these types of institutions, and they treat awide variety of illnessesand injuries. There maybe awide range of hospital sizes and types in larger cities. Particularly in North America, you could find hospitals thatoperate their wn ambulance service.

District

Usually the largesthospital in the area, a districthospital has manybeds available for critical care, acute care,and long-term care.

A particular kind of healthcare institution establised soon afterWorld War II in California is referred to as a "district hospital" in order tosolve the lack of hospital beds in many rural areas. District hospitals continue to bethe only publicly owned hospitals in 19 of California's counties. In the other nine counties, if one ormore ather haspitals are located at a significant distance from a local town, district hospitals remain the only locally accessible hospitals. District hospitals include twenty of California's critical-access hospitals and twenty of the state's rural hospitas. They are created by localgovernments, haveboards that are chosen by their respective communities, and arethere tomeet local needs. They play a crucial role in providing healthcare to patients who do not have insurance or who are enrolled in Medi-Cal, California's Medicaid program that serves low-income individuals, certain older citizens, people with disabilities, children in foster care, and pregnent women. In Califarnia,district hospitas provided uncompensated treatment worth $54 mllion in2012.

PECIALIZED

A children's hospital located in Auckland, New Zealand, is Starship Children's Health.

One or a small number of closely linked medical specializations are the main focus of a specialty hospital.[8] There are many different kinds of hospitals, each catering to a different population with unique medical requirements. For example,there are psychiatrichospitals that treat atients with mental health issues, cancer centers that treat cancer patients, orthopedic hospitalsthat treat broken bones, and so on.

FachkranenhausCoswig (thoracisurgery) is one example of a specialized hospital in Germany. In contrast to multispecialty hospitals, which house specialists in a variety of fields, specialty hospitals in India are referred to as super-specialty hospitals.

Comparedto normal hospital,specialized hospitas may assist in lowering healthcare expenses. For instance, the Bangalore cardiac unit of Narayana Health specialies in heartsurgery and can accommodate a much larger patient load. It is the biggest institution of its kindin the world, doing 3,000 pediatric cardiac procedures yearly, and has 3,000 beds. Because surgeons are paid a set salary rather than on an operation-by-operation basis, the hospital may benefit from economies of scale and lower procedure costs as the number of operations grows. Having each expert focus on a single process, such as a manufacturing line, may also increase their efficiency.

ORGANIZATION PROFILE

ABOUT THE HOSPITAL:

Established in 2003, ARADHANA Hospital's mission is to provide first-rate medical treatment to residents in forty-plus communities, in addition to JSW Group workers, associates, and their families. With its recent expansion and renovation, ARADHANA Hospital is now an ultra-modern facility with 157 beds, providing affordable healthcare. General medicine, obstetrics and gynecology, orthopaedics, general surgery, ophthalmology, ear, nose, and throat, dental care, and occupational health services are some of the important disciplines offeredby the hospital. The hospital provides round-the-clock emergency and trauma treatment, as well as pharmacy services, nursing care, andthe mostrecent, cutting-edge facilities. The hospital has eight operating rooms (four of which are modular), four emergency rooms, five observation rooms, eleven surgical intensive care units, and a dialysis unit with five beds.

Among the several medical centers overseen by the Narayana Health Group is the ARADHANA Hospital in Bellary. Those in need of medical attention in the Toranagallu district and the surrounding regions may get it at this hospital. The future of universal, high-quality healthcare lies with ARADHANA Hospital, as it does with all of the Narayana Health Group's multispecialty hospitals.

We are devoted to India's progress and will stop at nothing to ensure that all citizens, rich and poor alike, have access to quality healthcare. Our commitment to providing first-rate medical care remains unwavering. In order to provide patients with the greatest medical treatment possible, each hospital has invested in cutting-edge infrastructure and employs cutting-edge technology. Unparalleled medical care that goes out to worldwide standards is what we give. As the top urology hospital in Ballari, we treat difficult urological diseases like pros.

The top urologists and nephrologists are employed as Consultants to enhance our therapy. Known as one of Ballari's top cardiac hospitals, we provide ambulance services around-the-clock to facilitate emergency care for conditions including heart attacks, strokes, and accidents.

We also provide ambulatory and inpatient treatment as needed, as well as a diagnostic lab and pharmacy. We consistently have additional beds available for outpatient treatment to guarantee that patients get high-quality medical care. Not only that, but we also have an ICU, six OTs, one suite specifically for endoscopy, four critical care beds, and a specialized trauma care department. Call us right now to schedule an appointment if you want to get top-notch medical care.

DELIVERY OF SERVICES

The public and users alike see healthcare systems primarily via the provision of services at various levels, from basic care to secondary and tertiary care.[14] In today's society, there are several methods for giving medical treatment. A variety of settings, including homes, communities, workplaces, and hospitals, may serve as delivery sites. Direct patient-provider interaction, or "in the flesh," is the gold standard. In the majority of nations, this is how general medicine works. However, in absentia health care is on the rise because to advancements in telecommunications technology. In this context, "non-face-to-face communication" is any exchange between a healthcare provider and a patientthat doesnot take place in person.

The management and organization of services, as well as the incentives affecting providers and consumers, determine whether or not health care are accessible, covered, and of high quality. Such treatments are often covered by the patient or their health insurance in hospitals that operate under market-based health care systems, like the one in the US. Systems funded by the government (like the UK's National Health Service) are another option. A considerable

portion of the population in many developing nations has access to health care thanks to development assistance and other forms of charitable and volunteer-based finance.

The way that healthcare costs are structured might also differ significantly across nations. For example, in China, hospital fees often account for 50% of medicine costs, a significant portion of equipment costs, and a negligible portion of healthcare provider fees. China's healthcare sector underwent a long-term transition that started in the 1980s. Government contributions to healthcare expenses have decreased from 36% to 15% throughout the first 25 years of this change, with individuals bearing the majority of the responsibility for managing this decline. A few number of state-owned hospitals have also undergone privatization during this time. Foreign investment in hospitals up to 70% ownership has been promoted as a means of incentivizing privatization.

VISION, MISSION AND QUALITY POLICY

VISION:

To become the preeminent healthcare facility in the nation, to improve the quality of life for the people we serve, and to deliver world-class medical treatment to our area.

MISSION:

Our goal is to provide India's general public with high-quality, reasonably priced healthcare. The abbreviation "iCare" stands for "innovation and efficiency," "compassionate care," "accountability," "respect for everyone," and "excellence as a culture." Through the implementation of our business plan, we simultaneously want to produce a solid financial performance and provide long-term value to our shareholders.

QUALITY POLICY:

- Endorsed by
- JCI stands for Joint Commission International.
- Hospitals and healthcare providers are accredited by the National Accreditation Board (NABH).
- Security for patients.
- Program for infection control.

SERVICES

- 24/7 Emergency
- Laboratory Services: Biochemistry/ Clinical Pathology/ Haematology
- Pharmacy
- Ambulance
- Occupation Health Services

FACILITIES

- Adult Intensive Cardiac Care Unit
- Neonatal Intensive Care Unit
- Labour Room
- Exclusive Maternity Ward
- Pulmonary Function Test
- Computerised Eye Test
- Audiometry
- Non-invasive Cardiology: 2D Echo, ECG, TMT
- Radiology: Ultrasound, Colour Doppler, C Arm, 16 slice CT scan
- Dialysis Services

INFRASTRUCTURE FACILITIES:

- Library Consultant
- Pharmacy
- Dialysis-Unit
- Continues nursing-education
- Wi-Fi
- Cafeteria

SWOT ANALYSIS:

STRENGTH:

- Present in all major cities: This hospital's accessibility in all major Indian cities is one of its outstanding features. Thisguarantees that consuners will find it simpler to access the services from any location. Because the hospitals are conveniently located across the many cities, it is much simpler to take advantage of any services, even in an emergency.
- Highly skilled and seasoned medical professionals — a hospital's assets are its staff and amenities. This is somethingthat Apollo Hospital hasin stock. You may be confident that you are in excellent hands—not just callous ones—because every doctr who has been selected has the highest qualifications and the best experience. This is what sets this hospital apart from the vast array of others.
- The well-known brand name of Apollo institutions is another distinguishing feature that distinguishes it from competing institutions. Years of hard effort and devotion went into building the brand name.
- Another favorable part of Apollo Hospitals is their amazing and timely services. There is a special emergency room for the most serious and critical situations. Furthermore, it is organized into distinct divisions for each element, such as the renal, cardiac, gynecological, and cancer departments.

WEAKNESS:

- One of the drawbacks that has existed from the start is the high prices. Even for their top-notch services, they demand a high price that the average person may not always be able to pay. Because of this, even with the services they provide, this hospital is not usually the first choice for the general public.
- Absence of it in suburban regions: hospitals are mostly located in and around major cities, not in or near suburban areas. This is thereason why a lot of patients from outlying suburbs choose not to use the hospital's services.

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OPPORTUNITIES:

- The rising expense of healthcare in developed nations - the price hikes at Apollo Hospitals are stillfar lower than the average cost in developed nations. This is oneof the main reasons why this hospital is ahead of its time in termsof prospects.
- India is one example of a developing nation withlimited access to quality healthcare facilities. This is unquestionably a major factor that contributes to favorable chances for Apollo Hospitals. This hospital has a great opportunity due tothe lownumber of sophisticated healthcare service providers in the nation.

THREATS:

- Newer hospitals are altering their offerings in tandem with the advancement of technology and novel medical techniques. One of the main dangers that the Apollo Hospitals may face in the near future is this. In order to effectively serve patients with their enhanced and additional services, the majority of upscale hospitals are integrating newer technology into their offerings. This is one of those risks that this institution may encounter, but it will be difficult to remove given the level of brand trust that has grown up around it.

FUTURE GROWTH AND PROSPECTS:

- The establishment of a transplant unit for organs such as kidneys, hearts, and livers, as well as a comprehensive cancer center.

ANALYSIS OF FINANCIAL STATEMENT

Profit and loss account for ending 31 march 2022 & 31 march 2023

Particulars	Note. No	31st march 2022 In lakhs	31st march 2023 In lakhs
1.Revenues - operations (gross)	18	22,760.82	30,507.29
Less; Excise-duty			
Revenues, operation (Net)		519.60	1,037.36
		22,241.22	29,469.93
2.Other income	19	488.17	402.37
3.Total-revenues (1+2)		22,729.39	22,872.30
4.Expenses;			
(a) The price of the materials used;	20	2,467.38	3,301.13
(b) Variations in the completed products and work-in-progress inventories	21	745.78	(1978.99)
goods and work-in-progress			
(c) he expenditure of employee benefits	22	5,499.22	5,702.07
(d) The financing expenses	23	1.24	235.24
(e)Depreciation &amortization	8(c)	437.55	489.41
(f)Other expenses	24	13,493.72	20,006.33
Total expenses		**22,644.89**	**27,755.19**
5.Profit before exceptional items & tax (3-4)		84.50	2,117.11
6.Exceptional items	44	577.00	-
7.(Loss)/profit before tax (5-6)		(492.50)	2,117.11
8.Tax expenses/(benefits);			
(a) Current tax		390.00	1,000.00
(b) Deferred tax		540.43	(318.52)
		930.43	681.48
9.(Loss)/profit for year (6-7)		1,422.93	1,435.63
10.Earnings per equity share Rs 10 each;	36	**(16.26)**	**16.41**

SANJEEVANI HOSPITAL Balance sheet as at 31 march,2022

Equity and Liability	**Note No.**	**31st march,2022 (in lakhs)**	**31st march,2023 (In lakhs)**
1.Share holder's funds			
(a)Share-capital	2	875.00	875,00
(b)Reserves & surplus	3	36,169.67	37,662.67
2.Non-current liabilities			
(a) Other long-term liabilities	4	28,00	28,00
(b)Long-term provisions	5	1,696.44	1604,96
3.**Current liabilities**			
(a) Trade payables	26		
Total o/s dues to MSE'S		-	-
Total o/s dues to crts other than MSE's		1,920.53	1,675.53
(b)Other current liabilities			
(c)Short-term provisions	6	6,309.18	7,583.72
	7	611.16	609.33
Total liabilities		**47,609.98**	**50,258.97**
Assets			
1.Non-ccurrent assets			
(a)Fixed-assets			
Tangible assets	8(a)	7,486.95	7,416.54
Intangible assets	8(b)	281.92	-
Capital work in progress		138.99	374.88

(b)Non current assets	9	12,748.05	15,048.05
(c)Deferred tax-assets	37	2,160.00	2,700.43
(d)Long term loans & advances	10	7,885.81	7,392.46
(e)Other non-current-assets	11	332.90	1,323.60
2.Current assets			
(a)Current-investment	12	4,791.24	5,875.25
(b)Inventories	13	6,032.76	6,489.79
(c)Trade receivables	14	846.64	296.78
(d)Cash and hospital balances	15	2,175.46	1,344.30
(e)Short term loans &advances	16	1,814.48	2,024.28
(f)other current assets	17	914.36	2.63
Total Assets		**47,609.98**	**50,258.97**

FINANCIAL HIGHLIGHTS:

1.CURRENT RATIO:

Formula:

Current ratio =current assets/current liabilities

2021-2022= 136,86,143/34,22,438

=3.99

2022-2023=1,25,14,237 /21,14,237

=5.91

INTERPRETATION:

A review of the financial data for the years 2021–2022, showing a current ratio of 3.99, and 2022–2023, showing a current ratio of 5.91. The firm's strong liquidity position is shown by its current ratio of 3.99 and 5.91, which is lower than the conventional current ratio of 2:1.

2 QUICK RATIOS:

Formula:

Quick ratio= current assets- stock/current liabilities-bank overdraft

2019-2020= 1,36,86,143-65,34,781/34,22,438

= 2.08

2022-2023= 1,25,14,237-66,06,180/21,14,237

= 2.79

INTREPRETATION:

Financial analysis of liquidity assets a liability, where the quick ratio is the year 2021-2022is 2.08 and in year 2022-2023 is 2.79. The standard ratio is 1:1 the ratio is more than standard-atio hence it is good to the company.

3 CASH RATIOS:

Formula:

Cash ratio= cash equivalents/current-liabilities

2019-2020=39,56,751 /34,22,438

=1.156

2022-2023= 29,97,899/21,14,237

= 1.41

INTERPRETATION:

Financial-analysis for cash with current liabilities shows that the company's standard cash ratio should be 1:1, but in 2022–2023 it is 1.41 and in 2021–2022 it is 1.15. This means that the cash is more than the current liabilities. Therefore, the business can pay off all of its short-term debt.

4. RETURN ON ASSET-RATIO:

Return on assets ratio = Net profit /Total assets*100

2021-2022= 63.26M/86.25M*100

=73.34%

2022-2023 =99.82M/151.58*100

= 65.85%

Interpretation:

Using a return on assets of 73.34% in 2021–2022, and 65.85% in 2022–2023, we can calculate the net profit after assets. Ten percent is the industry average for return on assets. If a company's return on assets is greater than the industry average, it means that its total assets are profitable.

5. **DEBT RATIO:**

Total liability / Equity Shareholders

2021-2022= 35.67 / 28.59 = 1.24

2022-2023 = 36.86 / 28.59 = 1.28

INTERPETAION:

The debt ratio in the year 2021-2022debt ratio 1.24, in the year 2022-2023 the debt ratio 1.28, it's compering to both the year debt ratio is increased in 2022-2023.

CHAPTER-2

THEORETICAL FRAMEWORK

PATIENT SATISFACTION

A patient is someone who visits a doctor or dentist for regular checkups or other medical needs.

The degree to which a patient is pleased with the medical treatmnt they got fromtheir doctor is known as patient satisfaction.

Those whoget medicaltreatment are considered patients. Most of the time, the patient is sick or injured and need medical attention from a doctor, PA, APRN, vet, or another provider of health care. The opposite is true for in-patients, who are "admitted" to the hospital and remain there for an unspecified amount of time—often days or weeks—rather than just one night. Inpatient care is the term used to describe this kind of treatment. While checking into a hospital, paperwork is required in the form of an admission note; while checking out, paperwork is required in theform of a discharge note.

Maintaining a happy and loyal staff has always been a top priority for doctors. Productivity, innovation, and loyalty to the company all increase when workers are happy in their jobs.

To thrive in today's cutthroat health care market, where efficiency and quality are paramount, family doctors will need to establish practices that are conducive to attracting, motivating, and retaining talented staff. Plus, doctors could find that they're happier in their own jobs after all if they make the office a pleasant place to work for everyone.

Health plans risk not receiving full certification and losing the competitiveadvantage that coms withit if patient satisfactin levels are not high enough.

In the fieldof healthcare, patient happiness is "the degree to which the individual regards the health care service or product or the manner in which it is delivered by the provider as useful, effective, or beneficial."

This is part of the patient-physician interaction. Patient happiness and treatment quality, however, may not always coincide in the delivery of healthcare. In a nationally representative sample, a research found that "better patient satisfaction was related with more inpatient usage, higher total health care and prescription medication expenditures, and increased mortality, but less utilization of the emergency department. In a similar vein, patients tend to

be happier in private hospitals even when academic teaching institutions may provide superior results.

Surprisingly complicated, the idea of patient experience is often associated with patient happiness. The patient experience is being used as a metric to assess payment systems for quality in the healthcare industry as reimbursement and performancestandards have bcome more standard. But ther is stilla lot we don't know about the idea of the patient experience and how it affects how satisfid patientsare with their treatment. This article reviews a small selection of chosen literature to assist readers understand the complexity of measurement as it explores problems for measuring the patient experience, such as various contributing components and a lack of standard language.

HOSPITAL CUSTOMER SATISFACTION

Various challenges, such as competitive pressure, changing cost structures, and monitoring public and private groups, necessitate constant strategy reevaluation in the health care industry, which is currently experiencing fast growth and fierce competition among health care service providers. Because back then, it was commonknowledge that happycustomers are the key to a successful business.

In the subsequent years,researchers have made strides in measuring the characteristics of customer happiness and studying consumer behavior in relation to the services business. When customers have a positive experience with a service, their satisfaction level rises. Due to clients' increased knowledge and ability to research their alternatives, providingthem with healthcare services that meet their needs is more important than ever.

Consequently, clients play a significant role in health care services, necessitating a high level of skill and attention from service providers. The study delves into the topic of customer involvement in healthcare services and finds that, generally speaking, higher levels of customer involvement are linked to better performance in the unclear service dimension, but that low levels of customer involvement are sometimesassociated withlower levelsof predicted performance.

Executives in the healthcare sector have begun to paymore attentionto the problem of patient and customer satisfaction in recent years. Consequently, top executives in the field have been putting a lot of effort into projects aimed at increasing patient and consumer happiness.

Nevertheless, further study in this field is still required, despite their many endeavors and achievements (ACHE, 2006).

Customers are often dissatisfied with the parking facilities as they searchfor alternativesthat are more convenient and do not need as much walking. As a result, this is the first item on the list.

People do not like to spend toomuch timetrying to discover where they aremeant to be, hence lack of signage may also negatively effect customer satisfaction. Customers are not directly involvedin these factors, however patients demand clear indications of where they want to go.

Customers compare their experiences receiving treatments from one healthcare providerto another, which is another aspect. The real result ofthe treatment provided by the doctor, which helps the patient improve or get well, and the care or medicine they get are more likely to make the patient feel happy with their experience are two key elements in health customer satisfaction.

The reason behind the importance of these factors over others in India is that access mechanism quality, particularly in government hospitals, receivs lessattention thanother variables. The two dimensions of service quality are the technical quality and the functional quality. The former determines the customer happiness the most. The quality is distinct from other factors as it is an essential variable for effectiveness, which undoubtedly increases hospital production and gives the natural services principle efficacy. Cost and technology access mechanisms are improving; technology is saving time and yielding better outcomes than previously. The other is the cost of servicers; if patients believe they are paying the doctor for the services they receive, but therearefewer resources available to the public sector, revenue falls short of expenses and the capacity to provide the services to patients is reduced.

The most significant aspect influencing customer satisfaction is the attitude and treatmentthat patientsget from physicians, nurses, and other healthcare professionals. These factors may either please or disappoint people. Professional treatment that is kind, courteous, considerate, and sympathetic is what patients desire and should expect.

PATIENT SATISFACTION WITH HEALTH CARE

There is a thriving market for surveys that aim to gauge how satisfied patientsare with their healthcare providers. Between 1975 and 1979, there were 761 MEDLINE papers mentioning "patient satisfaction" as a key phrase. From 1993 to 1997, that number jumped to 8,505, a tenfold increase. Healthcare providersare ready to gauge their customers' responses by phone, fax, and modem, and consultants abound who market software packages that include patient satisfaction measurements into reports of hospital and health plan quality.

Proponents of patient-centered care, marketing experts, and consumer activists form an uncomfortable alliance on one side of the aisle. Skeptics argue that in this age with limited resources, our primary concerns should be with clinical outcomes like morbidity, mortality, and health status, rather than with patient happiness. They say that this concentration on satisfaction takes our eyes off of the real issues at hand. Some of these critiques are valid. Data on patient happiness is easier to get than indicators of technical excellence, and many healthcare organizations have given in to the temptation to stop there. Still, one of medicine's primary purposes is to facilitate patients' accomplishment of their objectives. Asking patients directly is the only certain approach tofind out whatthey want and whether their requirements are beingsatisfied since patients' values and aspirations vary greatly, can't be predicted from demographic and illness criteria alone, and are always evolving. Considering treatment "through the patient's eyes" is, according to this view, a professional and ethical need. It is not the importance of assessing patient happiness that often determines individual physicians, medical groups, hospitals, and health care results; rather, it is the reproducibility and meaningfulness of the measurements that matter most.

Many important measuring problems need to be resolved if patient happiness is to replace morbidity, mortality, and functional status. First, there must be clarity on what scale developers and end users are monitoring. A condensing of attitudes and perspectives, "patient satisfaction" is not a monolithic idea. Patients have perceptions of events. They capture the events. Values are the weights patients give such events. They show to whatextent patiens believ thatcertain events are anticipated, desired, or required.

THE IMPORTANCE OF PATIENT SATISFACTION

1) On average, happy patients will tell five others about their experience, whereas unhappy ones will tell nine or more individuals about their complaints. Thanks to the Internet, these views may be quickly and widely disseminated.

2. A patient's loss represents a significant financial loss due to the high expense of acquiring them. A patient might have found you via an insurance contract or an ad. In addition, you need to collect payment details and create a patient record for every new patient. A percentage of your investment in a patient is represented in every encounter with the patient, payer, physician's office, pharmacy, or laboratory, whether it's to confirm coverage, get patient records, physical exam results, etc., or to order or renew a prescription.

3. Research suggests that patient satisfaction and continuity of treatment are positively correlated, leading to improved patient outcomes. On the other side, if customers are unhappy and voice their grievances, it might lead to a decline in revenue or investment and even malpractice claims. Ensuring patient happiness is crucialfor several resons, inluding accreditation, company improvement, and risk management.

By sharing their experiences with care and fostering participation, patient satisfaction surveys may prvide patients a chance to take an active role in their treatment. Because most patients have lower clinical understanding than clinicians, the relevance of patient reporting has always been questioned. Reliability of patient reporting for specific parts of treatment from primary care and specialty physicians, as well as the growing socioeconomic significance of individuals actively participating in their own health care, may need a reevaluation of this perspective. Poor results and patient complaints and discontent in relation to each other The capacity of the patient to correctly identify their illness category.

Surveys of patient satisfaction offer physicians with immediate feedback and highlight areas for service improvement and risk reduction. Nonetheless, a lot of businesses and providers are unaware of how to make use of the patient happiness data they are given. This might be the case because healthcare professionals often ask patients to rate their experiences with them on a Likert scale or to answer yes/no questions.

MEASURING PATIENT SATISFACTION

Patient focused medicine is fundamentally patient satisfaction. Better treatment results are linked to higher patient satisfaction, which not only enhances the patient experience—something that every ill or wounded patient deserves. 2008 saw researchers show that better patient satisfactin was associated with better hospital treatment "for all conditions measured." Much more recent reearch has started to pinpoint the precise mechanism of this association. “Suggesting that patients are good discriminators of the typeof care they receive,” higher patient satisfaction has also been linked to lower in-patient mortality.

While better results are correlted with greatr patient satisfction ratings, many healthcare professionals still find it contentious to quantify patient happiness. Particularly physicians become irritated when groups start disclosing their patient happiness statistics to the public, including The Comonwealth Fund with their WhyNotTheBest website or the Ceters for Mediare and Medcaid Servics with their physician comparison website. Furthermore, working doctors typically worry when their reimbursement criteria include patient happiness.

There are cases when good medication could make patients less satisfied. Sometimes, contentment has more to do with getting what the patients want out of their therapy than with the level of care they get. As an illustration of this issue, consider the challenge that physicians have when they have solid informtion that ther patints are abusing drugs yet still want to provide them painkillers. Even if the doctor is doing their job well, the patient is probably going to be quite unhappy when they leave the clinic.

There is a ned to monitor patient satisfaction rates since, despite these uncommon situations, several research reveal a wide gap between patients' and physicians' perspectives on medical treatment. Since patients' and physicians' perspectives on individual episodes of treatment might vary significantly, it is impossible to provide an accurate picture of care quality without collecting data on patient satisfaction.

FACTORS INFLUENCING PATIENT SATISFACTION

The length and effectiveness of the patient's treatment, as wellas the doctors' empathy and communication skills, are the primry factors that dtermine how satisfied the patient is with their experience receiving medical care. It is supported by a positive doctor-patient rapport. Additionally, patients are often happier even when there is a longer wait time if they are aware of the steps required in a clinical visit and how long they should anticipate them to take. The level of work satisfaction that the caregiver experiences is another important elemet that affecs patient satisfaction [1].

BOTH IN- AND OUT-PATIENTS

A patient who stays in the hospital for fewer than twenty-four hours is called an outpatient, or out-patient. The patient remains registered even if they ae not formlly admittd with a note as an outpatient. The provider will typically provide a note outlining the reaon for the servce, procedure, scan, or surgery, along with the names, titles, andIDs of the particpating personnel, the patiet's name, date of birth, and ID, as well as the signed informed consent form. The note should also include the estimated time for pre-and post-sevice histoy and exam (befre and after), any necessary anesthesia or medications, and the estimated time of dischrge if there are no additional complications. This kind of treatment is referred to as ambulatory care. Occasionally surgery may be done without a formal hosptal admision or overnight stay. We refer to this as outpatient surgery. Among the numerous advantages of outpatient surgery include lower prescription drug costs and more effective use of the doctor's or surgeon's time. Rather of using an operating theater at a hospital, more surgeries are now being carried out in a surgeon's office, a practice known as "office-based surgery."For healthy patients having mild or intermediate treatments (limitd urologic, ophhalmologic, or ear, nose, and throat procedures, as well as procedues involving the extremities), outpatint surgery is the best option.

An in-patient, on the oter hand, is "admitted" to the hospial and stays overight or for an indeerminate time, usully several das or weeks, thouh in some extrme cases, such as with cma or persitent vegetatve state paents, stay in hospitls for years, sometims until deth. Treament providd in this fasion is calld in-patint care. The admision to the hopital involves the prouction of an adission note. The leavig of the hospial is oficially termed discharge, and

invlves a corresonding discharge note. Misdianosis is the leadig cause ofmedical eror in outptient failities. While patint safety eforts have focued on in-patent hospial setings for mre than a deade, mdical errors are een more ikely to hppen in a docor’s offce or outpatient clinic or center.

DAY PATIENT

The term "day patient" may refer to either an inpatient or outpatient who uses all of a healthcare facility's services but is not planning to spend the night. It was first used by mental health facilities to describe patients who needed help moving from inpatient to outpatient treatment. On the other hand, the phrase is increasingly often used to describe patients who go to hospitals for outpatient procedures.

PATIENT-CENTERED HEALTH CARE

A "day patient" is an inpatient or outpatient who makes full use of all the services offered by a healthcare institution but does not intend to stay overnight. First used by mental health institutions to characterize people requiring assistance transitioning from inpatient to outpatient care. Conversely, however, the term is being used more and more to characterize patients who vist hospitls for outpatient treatments.

IN-PATIENT CARE

Patients whose conditions need hospitalization are provided with in-patient treatment. Patients are now only admitted to hospitals in cases of serious illness or physical trauma, thanks to advancements in modern medicine and the proliferation of comprehensive outpatient clinics.

Hospitalization may be essential when a person's illness puts them in danger of harming themselves or others, even if they may not want to go to the hospital. It is usually ideal to seek assistance willingly if at all feasible, but if that isn't an option, it may be more compassionate than it seems to put your loved one or friend unwillingly in the hospital to obtain the care they need, particularly if there is a danger of harming themselves or others.

AIDET: FIVE STEPS TO ACHIEVING SATISFACTION

Sharp's team uses AIDET as a foundation for communicaton with both patints and their families and with one another. It is a siple acronym that stands for a highly effective method of speking with those who are often tense, fearful, and feeling exposed. Additionally, we may utilize it to interact with coworkers and other staff members, particularly when offering internal services.

1. Acknowledge

Smile and call individuals by name if you know them. One must have an attitude. Impress Ms. Jones in a way that will endure. "Good morning/afternoon. It's great to see you; we've been hoping you'd show up.

2. Introduce

Make courteous introductions to people. Describe yourself to them and how you plan to assist them. Instead of directing or gesturing, accompany individuals to their destination.

"Mrs. Smith, Dr. Hoegrefe will be visiting you today. He is a really good doctor. He is excellent at listening and responding to inquiries from patients. It's a blessing that he is your doctor."

"Mr. White, at our radiology department, Dr. Williams would want you to get an X-ray. Our staff of radiology technicians is highly skilled and use cutting-edge technology. I'm sure you're going to have an amazing time."

3. Duration

Maintain communication to reduce wait times. Inform everyone around you of any delays and the estimated duration of such delays. Dr. Heart had to respond to an emergency, so make it better and use service recovery procedures as needed. It might take up to thirty minutes before he can see you, but he wanted you to know that since he was worried about you. Would you want to set up an appointment for tomorrow, or are you okay with waiting?

4. Explanation

Tell them what you're doing, how the processes operate, and who to contact if they need help. Share any actons that they may need to take. Utilize words effectively. Inform pople

about wat you are doing, the processes involved, and who to contact in case they Speak, hear, and gain knowledge. Make time to assist. inquire, "Is there anything else I can do for you?"

5. Thank you

You shuld exprss your gratitude to someone. Encourage a grateful mindset. Appreciate the support, aid, or patronage of others. Utilize methods for reward and recognition.

We appreciate you going with Sharp. As your caregiver, it has been an honor.
Your call is appreciated. Could you possibly need anything else from me? The time is on my side [7]."

LITRATURE REVIEW

- A cognitive model was suggested by Priscilla et al. (2018) to evaluate the changing face of customer happiness or discontent in subsequent buying habits. In a three-stage longitudinal field research, they looked at five different product categories and discovered that satisfaction significantly moderates the relationship between intentions and actual behavior. Lagged intention influences repurchases of a certain brand, while unhappiness with brand consumption is more likely to influence switching behavior, according to their findings.
- The work of David and Wilton (2018) adds to the existing literature on consumer satisfaction. They use a model initially put forth by Churchill and Surprenant to study the impact of perceived performance, how appealing ideas of comparison standards and disconfirmation can be used to understand the process of satisfaction formation, and whether or not there are multiple comparison processes involved in satisfaction formation. According to their findings, contentment is impacted not just by predicted performance and subjective disconfirm, but also by perceived performance.
- In 2018, Saha attempted to study the connections between health, life satisfaction, work satisfaction, and life satisfaction across time. There was also an investigation into the connections between these four factors and biographical ones. Nurses in Nigeria were the subjects of the research. Since responses from retirees and part-time nurses varied in terms of work satisfaction and other criteria, the data was only gathered from full-time personnel.
- A methodology for evaluating service levels, overall quality, and value was put up by Bolton and Drew (2018) based on consumers' previous experiences and expectations. Local telephone service consumers in their homes were the focus of the model's application. Customers' ability to synthesize their impressions of a service into a final rating was the focus of their research. To determine what factors contribute to customers' opinions of a service's quality and value, they created a multi-stage model. The model laid out the chan reation that begn with he customer's expectations and ended with their evaluation of the service's quality and vlue, taing into accont thir impressions of the service's present performance and any disconfirmation experiences they may have had.

- According to Boulding et al. (2019), client retention is indirectly related to service quality. The author has provided a theoretical framework for understanding how service quality affects certain actions that indicate whether clients stay with or leave a business. The study's findings provide compelling proof that service quality influences them. Distinct variations in service quality are also shown by the results.
- In order to assist hospital management in developing marketing strategies to attract clients and grow their customer base, Aurora and Malhotra (2019) conducted a comparative investigation of the customer satisfaction levels of pblic and privae secor hospitals.
- In the context of price comparison advertising, Grewal et al. (2019) extended and incorporated earlier price perceived value models. To be more precise, the conceptual model explains how buyers' online reference prices, quality perceptions, acquisition value, transaction value, and purchase and search intents are impacted by advertised selling and reference prices. The conceptual model is tested in two experimental tests. The combined and individual finings from thee two studes provide credence to the theory that customers' internal reference prices are impacted by their perceptions of the quality of the products as well as the selling and reference prices that are promoted. The autors also discover that purchasers' views of transaction value acted as a mediating factor in the relationship between the stated selling price and acquisition value. Furthermore, purchasers' views of acquisition value acted as a mediator between the impacts of perceived transaction value and their behavioral intentions.
- Voss (2019) looked into how satisfaction in service exchange is determined by pricing, performance, and expectations. Expectations have an assimilation impact on performance and satisfaction ratings when price and performance are constant; expectations have no influence on these metrics when price and performance are discordant. The authors use information from a multimedia experimental design to estimate a contingency model they create to investigate these difficulties. In summary, the results validate the contingency paradigm and give empirical support for normative recommendations that advocate for setting reasonable performance standards and providing service guarantees that may be refunded.
- Garbarino and Johnson (2020) examine the connections between customer happiness, trust, and commitment, as well as their perspectives about satisfaction and future plans, for patrons of an off-Broadway repertory theatrical group in New York. Overall

satisfaction is the main mediating construct between component attitudes and future intentions for relational customers (individual ticket buyers and infrequent subscribers), while trust and commitment—rather than satisfaction—are the mediators between component attitudes and future intentions for high relational customers (consistent subscribers).

- A research on patent satisfction with priate health care facilities' outdoor services was conducted by Sharma and Chahal (2020). To find ot how satified patints were wth the diagnostic services they received, they conducted a survey. To gauge patient happiness, they have created a unique tool. The device records atmospherics, administration quality, and physician and medical assistant conduct. Considerations for the role of graphic characters include gender, employment, income, and education. They also recommended strategic steps based on their results to better serve the requirements of patients in the private health care industry. The study's recommendations included being more amiable and sympathetic to patients' issues, keeping the units tidy on the inside as well as the outside, reporting on patients' progress on a reglar basis ithout wating for them to ask, conducting surveys to fid out ow patiens feel abot the staff, and implementing patient-centered policies and procedures.
- According to Simester et al. (2020), a global corporation in the US and Spain use cutting-edge techniques to develop and execute programs aimed at improving customer satisfaction. The results of their tests paint a complicated and unexpected picture, drawing attention to implementation problems, a concept of residual satisfaction that is not represented by client wants, and the need for managers to combine nonequivalent controls and nonequivalent dependent variables.
- Marketing strategy creation relies heavily on consumer assessments of quality and happiness, according to research by Ofir and Simonson (2021). It is becoming more and more customary to ask for such reviews, so customers usually anticipate being asked for them after purchasing a product or service. A number of empirical investigations, both in and out of the lab, show that consumers are less likely to buy and promote the assessed services, and that their ratings of those services' quality and satisfaction suffer as a result. Expected judgments are biased negatively even when encouraged to evaluate both the positive and negative features; this bias is evident whether the actual quality is high or low.

- In their study, Dholakia and Morwitz (2021) looked at how quantifying satisfaction affects customer behavior in the long run and how wide that influence is. In a financial services context, researchers discovered that assessing customer happiness altered both one-time purchases and relational customer behaviors, with impacts that persisted for months and even a year after the first measurement. Concerns about the ethics, methodology, and interpretation of applied marketing research studies were prompted by their findings.
- According to Sharma and Chahal (2022), hospitals now prioritize patient happiness because people are more aware of the importance of healthcare. The authors looked at what makes Indian government outpatient services satisfying for patients. According to their research, patient satisfaction is affected by four main factors: the conduct of physicians and medical assistants, the quality of the environment, the efficiency of the administration, and the conduct of the actual treatment itself. In order to address the demands of developing country patients in the public health care system, they also offered strategic measures.
- In their research, Folkes and Patrick (2022) found that consumers' opinions of service providers were positively impacted. Customers who have never dealt with the service before are more likely to generalize good or negative experiences with one employee to the rest of the company's service providers when they hear good things about one employee. There were four studies that looked at different amounts of data on the service, the company, and the provider. Despite variations in methodology and outcome measures between research, the positive impact was still validated.
- Customer retention and share development over time were examined by Vernoer (2022) in connection to relationship marketing tools and customer relationship perceptions. When comparing two time periods, customer share development refers to the change in customer share, while customer relationship perceptions are assessments of the strength of the connection and the supplier's products. According to the findings, direct mailings have an effect on customer share development, but emotional commitment and loyalty programs that provide financial incentives have a good effect on client retention as well.
- A theoretical framework was established by Anderson et al. (2023) that explains how future customer behavior and, therefore, the amount, timing, and risk of future cash flows are influenced by past customer pleasure. They discover, empirically, that

shareholder value and consumer pleasure are positively correlated. Significant difference is also seen across companies and sectors.

- In their research of the customer relationship management process, Reinartz et al. (2023) concluded that satisfying consumers requires maintaining positive relationships with them. In this work, they (1) conceive and describe a construct of the CRM process and its dimensions; (2) operationalize and verify the construct; and (3) empirically examine the effects of CRM process implementation on organizational performance. Two cross-sectional studies covering three countries and four distinct sectors answer their research questions. The major result is a beginning, maintenance, and termination of a CRM process measure that is theoretically sound.
- Homburg and colleagues (2023) support a nonlinear, functional structure based on disappointment theory and show the existence of a strong, positive impact of customer satisfaction on willingness to pay through two experimental studies (a lab experiment and a study involving a real usage experience over time). The second, too, looks at dynamic elements of the connection and offers proof that cumulative satisfaction has a greater influence on readiness to pay than satisfaction unique to a particular transaction.
- Customer knowledge and satisfaction are assessed by Mithas et al. (2023). They demonstrate by analyzing historical data from a cross-section of American companies that using CRM software is favorably correlated with increased customer satisfaction and knowledge. They also discovered that companies which share their customer-related data with their supply chain partners improve their understanding of their customers.

GAP ANALYSIS

Customer satisfaction has been the subject of a great deal of research. We have made an effort to provide a concise literature analysis on customer satisfaction in general and customer satisfaction from medical services in particular.

CHAPTER-3
RESEARCH METHODOLOGY

This chapter deals with research methodology selected by investigator in order to conduct the study entitled "A study on Patient Satisfaction In ARADHANA HOSPITALS , Ballari.''

STATEMENT OF THE PROBLEM

In order to promote healthy living and address the health problems that people experience for a variety of reasons, hospitals have been developed. In order to deliver timely services, hospitals are essential. However, the government hospitals do not currently offer services of the anticipated quality because they lack the infrastructure and facilities needed to deliver care promptly. The main causes of treatment delays include a lack of infrastructure, the absence of cutting-edge medical facilities, and a shortage of doctors, nurses, and staff. The patient-doctor interaction has evolved a partnership between a consumer and a service provider as a result of corporatization of the medical industry today and the rise in patient standards.

The entire study emphasises that factors like tangibility, assurance, empathy, and accountability can be used to gauge service quality. The majority of research are focused on the level of care provided in private hospitals and overseas. The researcher has made an effort to use the already available dimensions to gauge the level of service provided in general hospitals.

OBJECTIVE

1) A study to evaluate in-patient satisfaction in ARADHANA HOSPITALS .
2) To suggest methods to improve the patient satisfaction.
3) To measuring the satisfaction level of patients' in ARADHANA HOSPITALS.
4) To study patients' expectations towards facility or service provided by hospital
5) To study perceptions from the hospital services.
6) To measuring their satisfaction level from the hospital services. For both the first and second objective of study i.e.. the patient' expectations and their perceptions of hospital services.

RESEARCH APPROACH

The research approach adopted in the study was qualitative approach. It includes collection of information's, opinions and attitudes directly from the subject of the study through questionnaire schedule. Type of research adopted was descriptive. As it was a fact finding study.

TYPE AND SOURCE OF DATA

There are 2 types of data: primary data and secondary data.

Secondary Data: To suggest solutions to the problems observed during the survey will be done through secondary data.

Primary Data: Primary data will collect from respondents. The respondents will either the patients themselves or their relatives. For sample selection, a multistage sampling procedure will be followed. The information will collect through a structured questionnaire.

Tools Used: Excel

Sampling Method: For sample selection, a multistage sampling procedure will be followed. At the first stage, sample units consisted of the total number of general wards and private wards in the hospital.

Sample size: 20 patients & their relatives from General Wards & 15 patients & their relatives from Private Wards

Target Audience: Patients & Doctors of ARADHANA HOSPITALS

TOOLS AND TECHNIQUES

The tool used for the study was questionnaire. The questionnaire divided into two parts. Part-A consists of demographic data. Part-B consists of questions related to in-patient satisfaction.

SI NO.	ITEMS	QUESTIONS	NO.OF QUESTIONS
1	Questions related to demographic data	1,2	2
2	Questions related to the satisfaction of treatment	3,18,19	3
3	Questions related to staff	4,5,6,8,9,10,14	7
4	Questions related to facilities provided	7,11,12,13,15,16,17	7
5	Questions related to the needs of patient	20	1

PLAN OF ANALYSIS

The collected data analyzed using tables and graphs and in simple percentages.

HYPOTHESIS

H:0 patients expectation would be the determining factor for any customer satisfaction.

H:1 patients Perception would be another major of benchmarking the hospital service as per the customer needs.

LIMITATION

- This project covered the ARADHANA HOSPITALS aspects of study
- This project has covered Ballari as a region as the duration of the project only 2 to 3 weeks. So I have to collect and finish this data within the time.

CHAPTER-4

DATA ANALYSIS AND INTERPRETATION

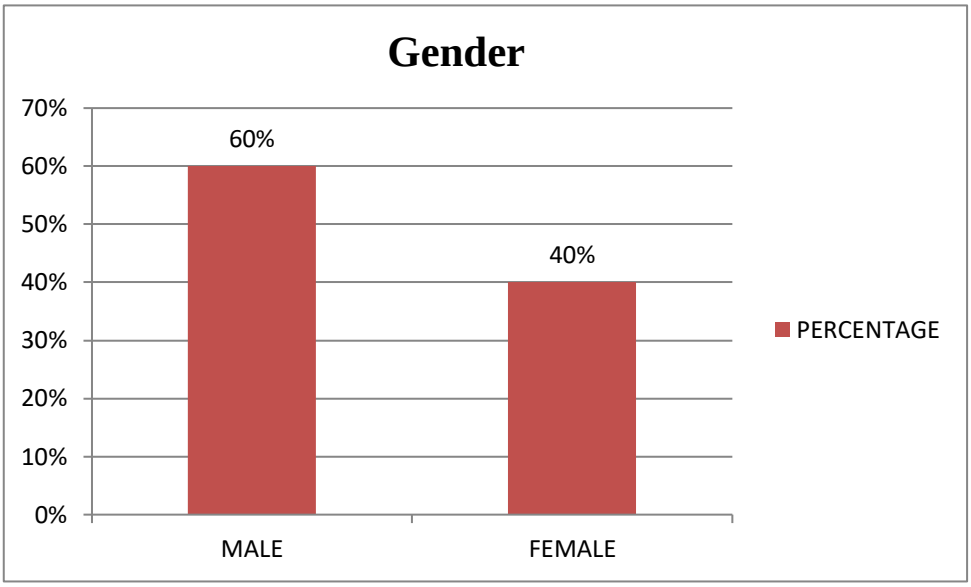
TABLENO: 1

Frequency table showing gender of respondents.

RESPONSES	FREQUENCY	PERCENTAGE
Male	60	60%
Female	40	40%
Total	100	100%

ILLUSTRATION NO: 1

Diagram showing gender of respondents.



The above diagram revealed that 60% of respondents were male and 40% were female.

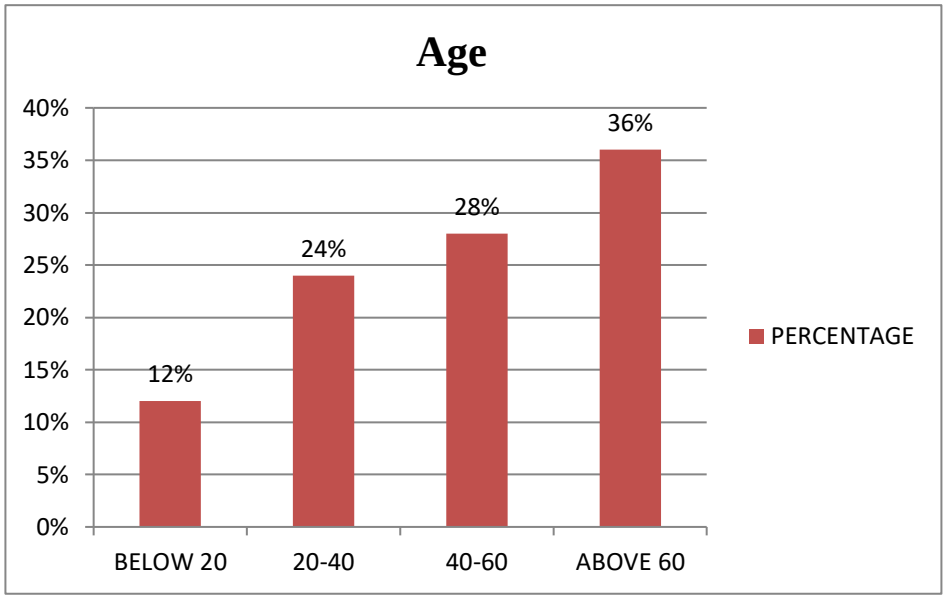
TABLE NO: 2

Frequency table showing age wise distribution of respondents.

RESPONSES	FREQUENCY	PERCENTAGE
Below 20	12	12%
20-40	24	24%
40-60	28	28%
Above 60	36	36%
Total	100	100%

ILLUSTRATION NO: 2

Diagram showing age wise distribution of respondents.



The above figure shows that 12% of the respondnts were in age group below 20, 24% were in age group 20-40, 28% were in age group 40-60and 36% were in age group above 60 years.

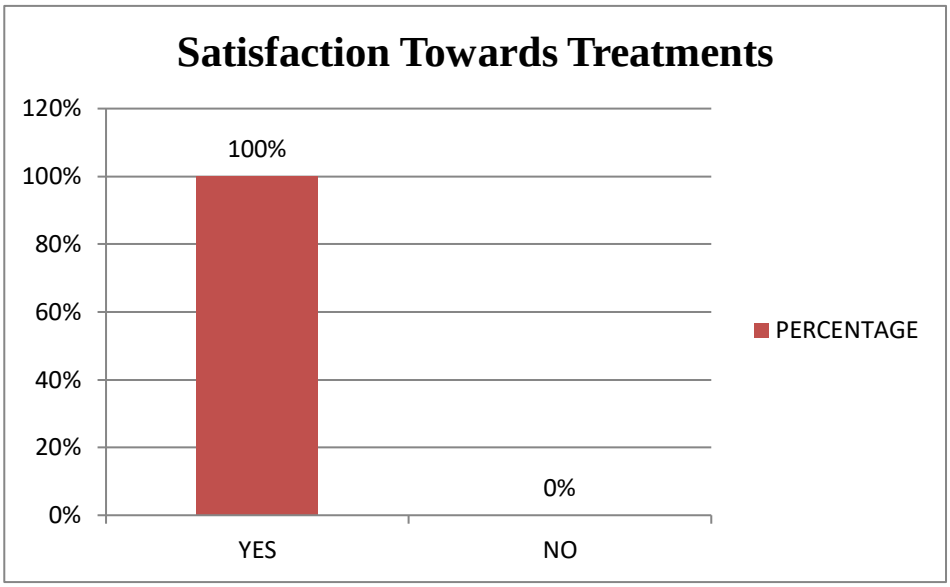
TABLE NO: 3

Frequency table showing respondnts opinon about satisfaction with treatments given.

RESPONSES	FREQUENCY	PERCENTAGE
Yes	100	100%
No	0	0%
Total	100	100%

ILLUSTRATION NO: 3

Diagram showing the opinion about the satisfaction with treatments given.



The above figure revealed that 100% of the respondents were satisfied with treatment provided in the hospital.

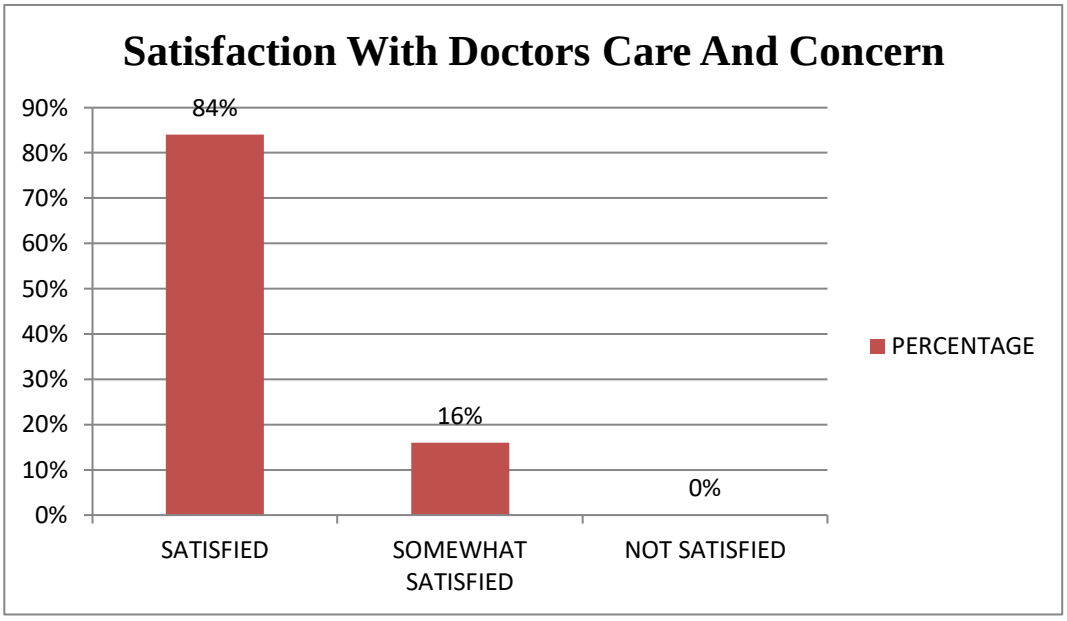
TABLE NO: 4

Frequency table showing whether the patients are satisfied with doctor's care and concern.

RESPONSES	FREQUENCY	PERCENTAGE
Satisfied	84	84%
Somewhat satisfied	16	16%
Not-satisfied	0	0%
Total	100	100%

ILLUSTRATION NO: 4

Diagram showing whether the patients are satisfied with doctor's care and concern.



The above figure revealed that 84% of respondents were satisfied with doctor's care and concern and 16% of respondnts were somehat satisfed with doctor's care and concern.

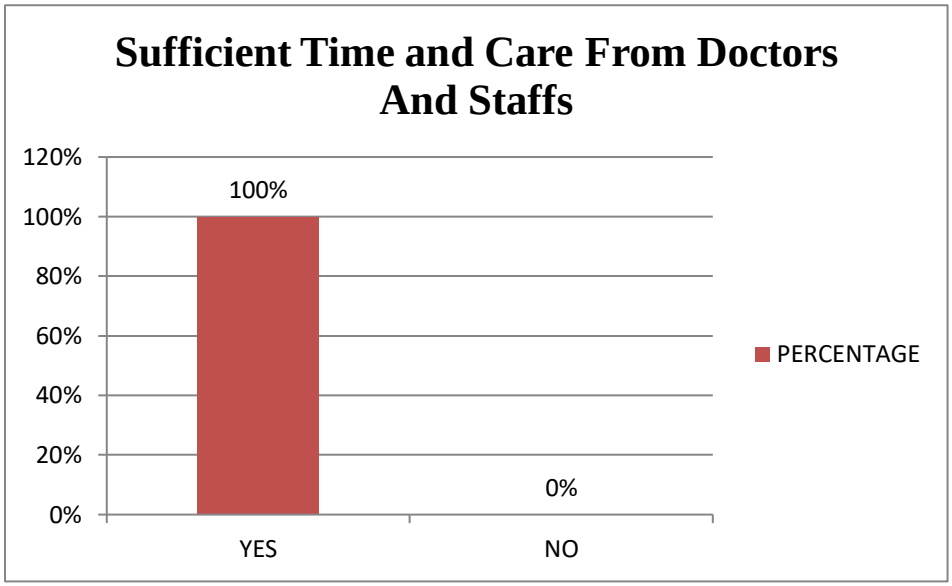
TABLE NO: 5

Frequency table showing that the opinin of the respndents about the time and care from doctors and staffs.

RESPONSES	FREQUENCY	PERCENTAGE
Yes	100	100%
No	0	0%
Total	100	100%

ILLUSTRATION NO: 5

Diagram showing that the opinon of the respodents about the time and care from doctors and staff.



100% of the respondents said that were satisied with the care given by the staff and the time spent for the patient care.

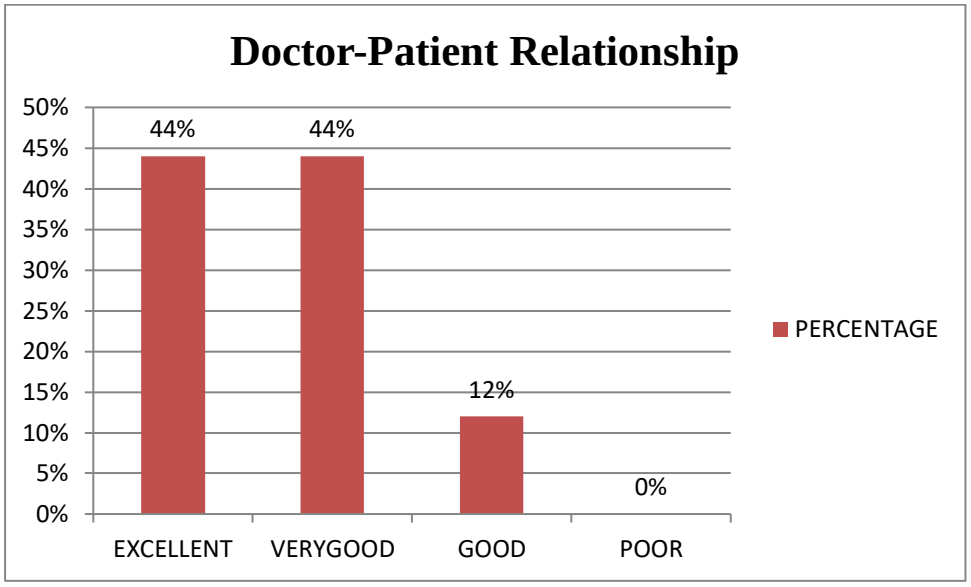
TABLE NO: 6

Frequency table showing the respondents rating on doctor-patient relationship.

RESPONSES	FREQUENCY	PERCENTAGE
Excellent	44	44%
Very good	44	44%
Good	12	12%
Poor	0	0%
Total	100	100%

ILLUSTRATION NO: 6

Diagram showing the respondents rating on doctor-patient relationship.



The figure shows that, 44% of the respondents rated doctor-patient relationship as excellent, another 44% rated it as very good and 12% rated it as good.

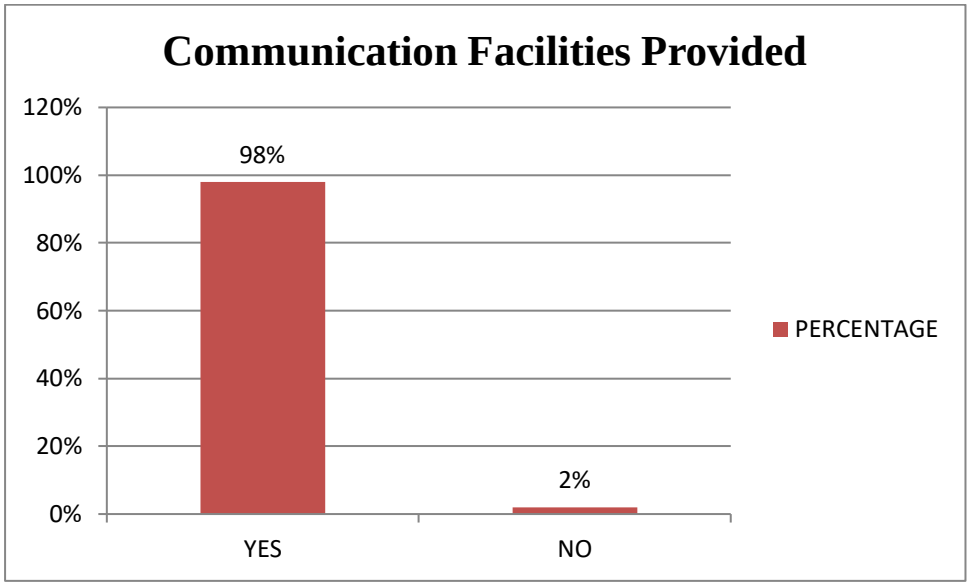
TABLE NO: 7

Frequency table showing whether the patients are satisfied with the communication facilities provided in hospital.

RESPONSES	FREQUENCY	PERCENTAGE
Yes	98	98%
No	2	2%
Total	100	100%

ILLUSTRATION NO: 7

Diagram showing whether the patients are satisfied with the communication facilities provided in hospital.



The above figure revealed that 98% of the patients were satisfied with the communication facilities provided in hospital and 2% not at all satisfied.

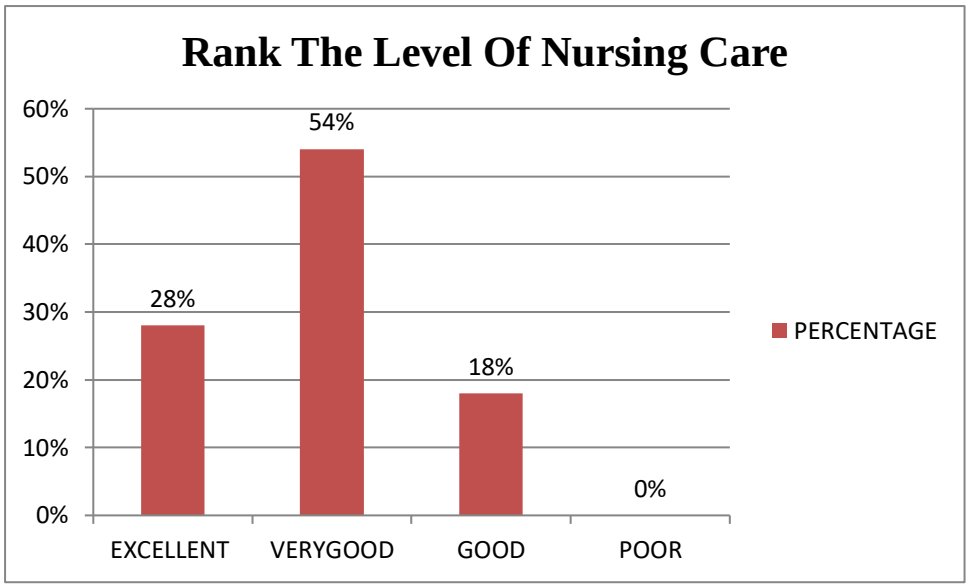
TABLE NO: 8

Frequency table showing the rating of nursing care.

RESPONSES	FREQUENCY	PERCENTAGE
Excellent	28	28%
Very good	54	54%
Good	18	18%
Poor	0	0%
Total	100	100%

ILLUSTRATION NO: 8

Diagram showing the rating of nursing care.



The figure revealed that 28% of the respondents opined that the level of nursing care was excellent, 54% opined that it was very good and 18% opined that the level of nursing care was good.

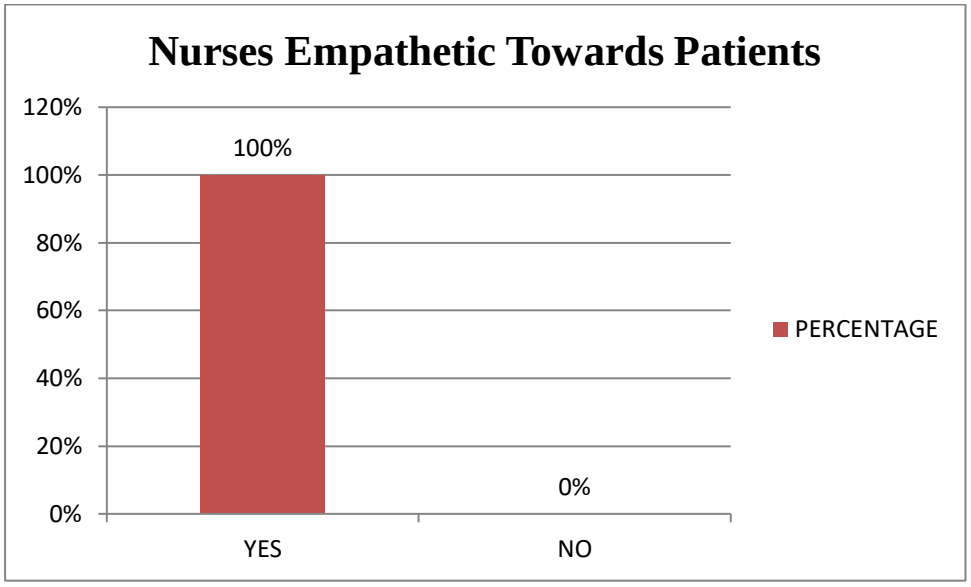
TABLE NO: 9

Frequency table showing whether the nurses are empathetic towards patients.

RESPONSES	FREQUENCY	PERCENTAGE
Yes	100	100%
No	0	0%
Total	100	100%

ILLUSTRATION NO: 9

Diagram showing whether the nurses are empathetic towards patients.



All the respondents opined that nurses were empathetic towards patients.

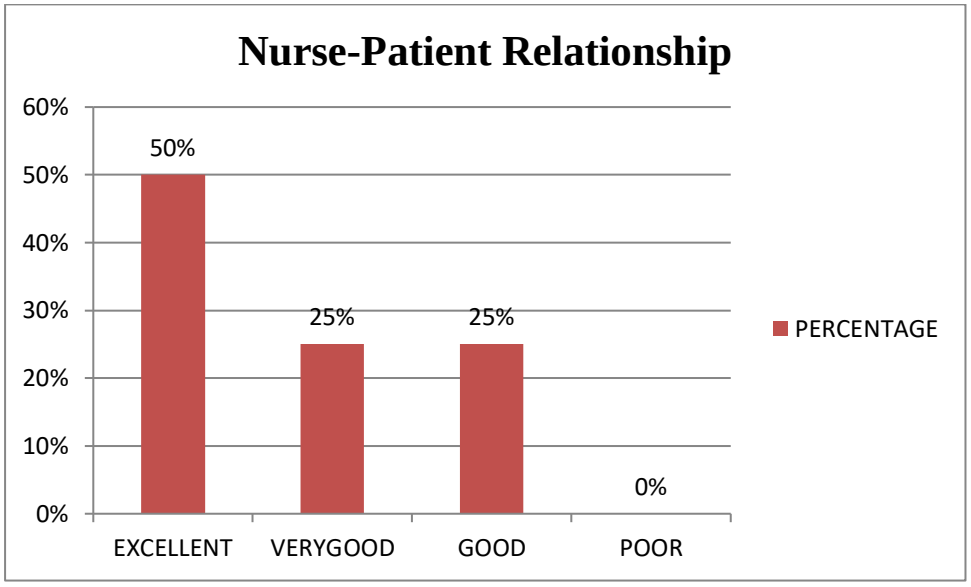
TABLE NO: 10

Frequency table showing opinion about nurse-patient relationship.

RESPONSES	FREQUENCY	PERCENTAGE
Excellent	50	50%
Very good	25	25%
Good	25	25%
Poor	0	0%
Total	100	100%

ILLUSTRATION NO: 10

Diagram showing opinion about nurse-patient relations.



50% opined that the nurse patient relationship was excellent, 25% opined that it was very good and another 25% opined that it was good.

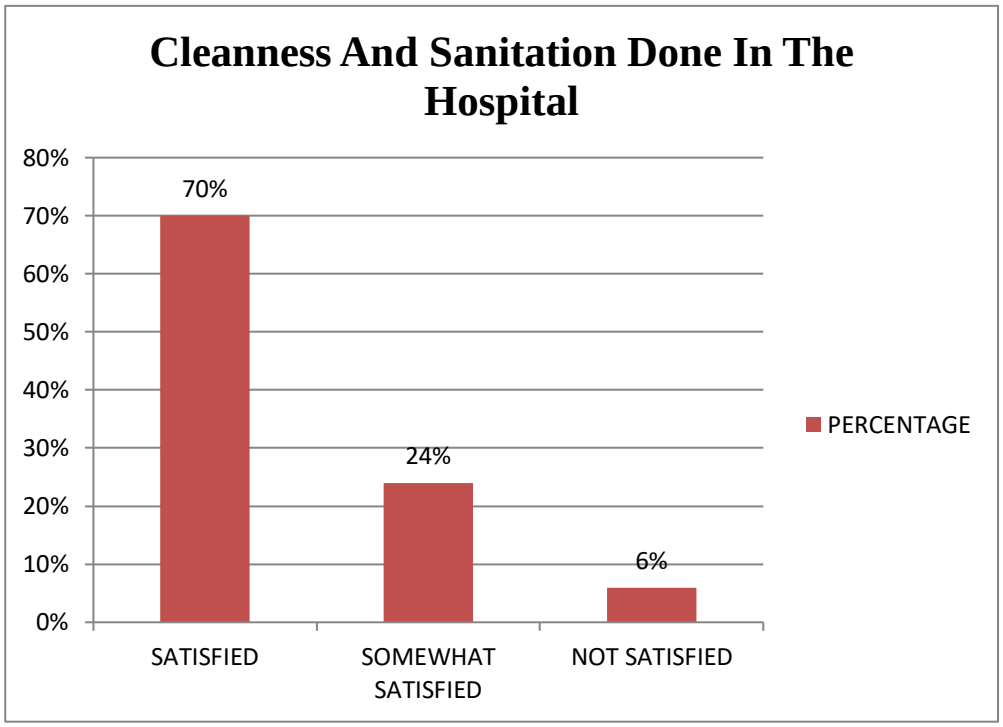
TABLE NO: 11

Frequency table showing whether the patients are satisfied with cleanness and sanitation provided in the hospital.

RESPONSES	FREQUENCY	PERCENTAGE
Satisfied	70	70%
Somewhat satisfied	24	24%
Not-satisfied	6	6%
Total	100	100%

ILLUSTRATION NO: 11

Diagram showing whether the patients are satisfied with cleanness and sanitation provided in the hospital.



70% were satisfied, 24% were somewhat satisfied, and 6% were not satisfied with cleanness and sanitation provided by hospital.

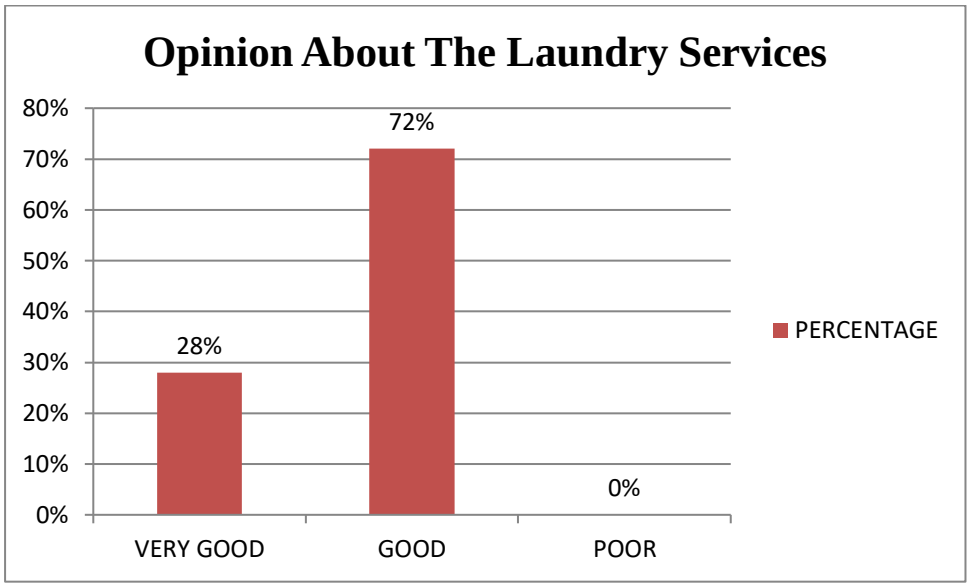
TABLE NO: 12

Frequency table showing the opinion about the laundry services.

RESPONSES	FREQUENCY	PERCENTAGE
Very good	28	28%
Good	72	72%
Poor	0	0%
Total	100	100%

ILLUSTRATION NO: 12

Diagram showing the opinion about the laundry services.



The above figure shows that, 28% of respondents rated laundry services as very good & 72% rated it as good.

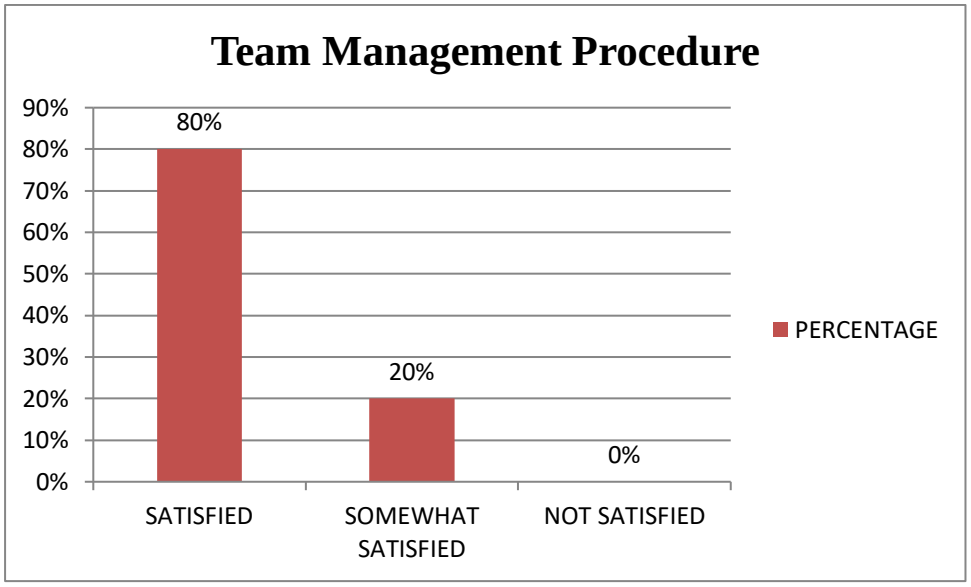
TABLE NO: 13

Frequency table showing the satisfaction with team management procedure involved in health care process.

RESPONSES	FREQUENCY	PERCENTAGE
Satisfied	80	80%
Somewhat satisfied	20	20%
Not-satisfied	0	0%
Total	100	100%

ILLSTRATION NO: 13

Diagram showing the satisfaction with team management procedure involved in health care process.



80% were satisfied with the team management procedure involved in health care process & 20% were somewhat satisfied.

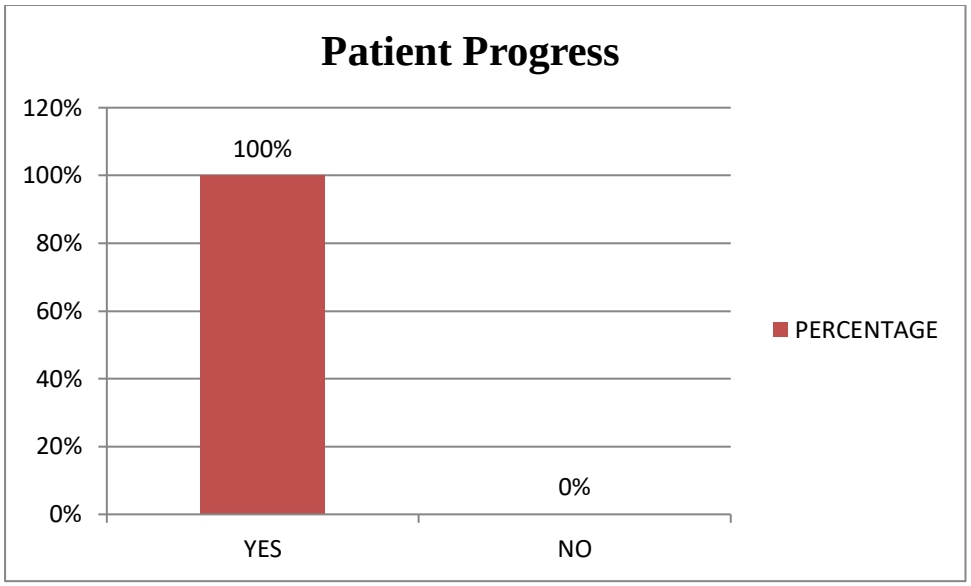
TABLE NO: 14

Frequency table showing whether the physician explain the patient progress regularly.

RESPONSES	FREQUENCY	PERCENTAGE
Yes	100	100%
No	0	0%
Total	100	100%

ILLUSTRATION NO: 14

Diagram showing whether the physician explain the patient progress regularly.



All the respondent opined that the physician explain their progress regularly.

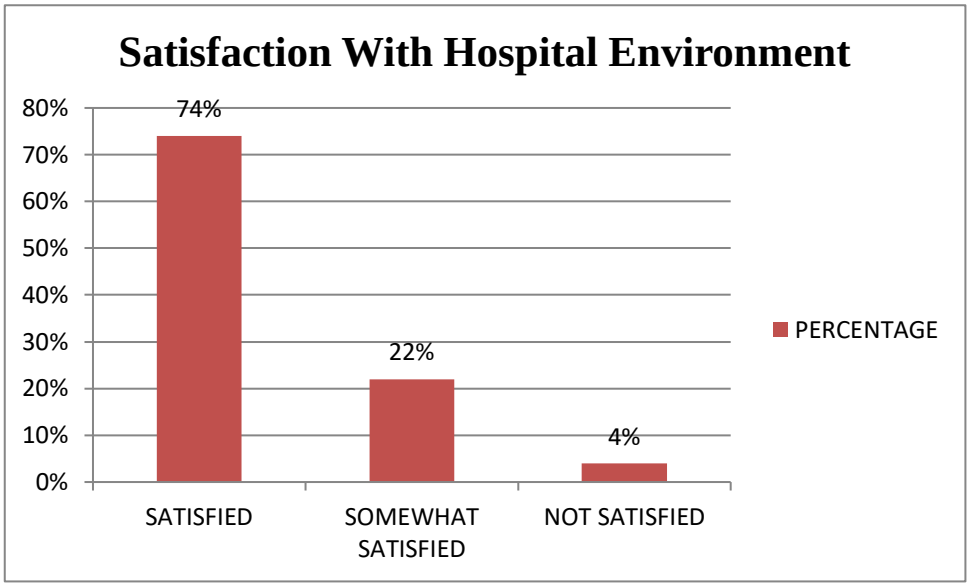
TABLE NO: 15

Frequency table showing whether the respondents are satisfied with the hospital environment related to the health care.

RESPONSES	FREQUENCY	PERCENTAGE
Satisfied	74	74%
Somewhat satisfied	22	22%
Not-satisfied	4	4%
Total	100	100%

ILLUSTRATION NO: 15

Diagram showing whether the respondents are satisfied with hospital environment related to the health care.



74% were satisfied, 22% were somewhat satisfied and 4% are not satisfied with hospital environment related with health care.

CHAPTER-5

FINDINGS

The study revealed that,

1) 60% of the respondents were male.
2) 36% of the respondents were in age group above 60 years.
3) All the respondents were satisfied with the treatment provided in the hospital.
4) 84% respondents were satisfied of doctor's care and concern.
5) 100% of the respondents were satisfied with the care from doctors and staff and the time they spent for the patients.
6) 44% of the respondents rated doctor-patient relationship as excellent and another 44% rated it as very good.
7) 98% of the patients were satisfied with the communication facilities provided in hospital.
8) 54% opined that the level of nursing care was very good.
9) All the respodents opied that nurses were empathetic towards them.
10) 50% of respondents rated nurse-patient relationship as excellent.
11) 70% opind that they are satisfied with cleanness and sanitation facility.
12) 72% opined that laundry services were good.
13) 80% were satisfied with the team management procedure involved in health care process.
14) All respondents opined that the physician explain their progress regularly.
15) 74% of respondents were satisfied with hospital environment.
16) 94% of respondents enjoyed the faciities provied by the hospital.
17) 98% of the respodents had positie attitde towrds hospital.
18) 72% of respondents opined that curative care treatment should be given the first priority.
19) 82% were satisfied with the investigation, treatment and service provided by the hospital.
20) 74% were satisfied with the physical, mental, social and spiritual needs.

RECOMMENDATIONS

1) The level of nursing care should be improved.
2) It is better to increase the number of in-patient rooms for providing efficient treatment.
3) Increase the number of nurses in all shifts, for providing better care to in-patients.
4) Patients are having trouble obtaining information on their doctors' data, despite the fact that the outpatient department's registration facilities are conveniently located between the entrance and the doctors' cabins.
5) Patients have a hard time getting to their doctors since the outpatient department is on the first level and the only way to get there is via the stairway.
6) When a patient has to return to the doctor for a diagnostic procedure, that's when they experience a waste of time.
7) The patient must be escorted by their physician to the ground level for the admittance procedure, which is both inconvenient and time-consuming for the patient.
8) Patient information centers may be strategically positioned at the entry to provide assistance and direction.
9) Equipment shortage causes extra time to be consumed
10) The physician's recommendation of diagnostic tests significantly lengthens the time it takes for the admissions department to transfer patients.
11) Patient information centers may be strategically positioned at the entry to provide assistance and direction.
12) Patients have less incontinence if the admission department is moved closer to the outpatient department.
13) Provision of necessary tools and machinery.
14) Making a system that only patients may use
15) In a systematic manner, giving patients their serial numbers at the counters prior to diagnostic procedures

CONCLUSION

The study entitled in-patient satisfaction in Aradhana Hospital was done to evaluate the satisfaction of in-patients and to suggest methods to improve the in-patient satisfaction.

The researcher found that majority of the patients satisfied with treatments, doctor's care and concern, nursing services, hospital environment and other facilities provided by hospital.

Researcher concluded the study by forwarding some suggestions such,

1. The level of nursing care should be improved.

2. It is better to increase the number of in-patient rooms for providing efficient treatment.

3. Increase the number of nurses in all shifts, for providing better care to in-patients.